

WEEK 11 · CAREGIVING

CLASS 1

Keeping and Caring

READING 1

Excerpt from the Introduction (pp. 1–14) and “Chapter 2: Residence and Relationships” in *Children As Caregivers* – Hunleth

Children As Caregivers: The Global Fight Against Tuberculosis and HIV in Zambia | Hunleth

Week 11, Class 1

Introduction — pp. 1–14

Chapter 2: Residence and Relationships — pp. 47–77

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Introduction

Maureen Nkhoma opened her eyes as I walked into her yard. Grabbing ahold of the well-worn quilt that covered her, she raised her head to acknowledge me. Her children, eight-year-old Loveness and twelve-year-old Bwalya, were hanging laundry nearby. She directed them to prepare the sitting room in their three-room house. “Outside is no good for a visit,” she told me, as the wind swept dirt from the bare ground. It was a crisp August morning in Zambia’s capital city, Lusaka.

“Get in,” Maureen said, when she saw me hesitate at the door to her house. I had been watching her as she rose slowly on legs weakened from extended sickness and disuse. She walked toward the house with her gaze to the ground. Each uncertain step she took was filled with an effort that, I realized only later, she did not want me to see. At her insistence, I went into the sitting room ahead of her and took a seat across from Loveness and Bwalya. We sat in silence until Maureen joined us.

Maureen had tuberculosis (TB), a diagnosis that came after months of weight loss, fever, and night sweats, and a cough that had lingered for much too long. For the past thirty years, TB has ranked among the most pressing infectious diseases in the world, its presence and deadliness driven by political upheaval, public health neglect, poverty, and under-resourced healthcare systems, and made so much worse by the emergence of the HIV epidemic.¹ Globally, an estimated 9 million people become sick with and 1.5 million people die from the disease each year (UNOPS 2015).²

TB transcends national borders.³ Globalization has increased the mobility of people, and complicated notions that TB—or any infectious disease—will remain confined to a particular nation or region. At the same time, TB offers a stark example of how global politics and policies have carved up the world in unequal ways, structuring who suffers from infectious diseases, who gets treatment, and

who recovers.⁴ Ninety-five percent of the deaths attributed to TB, for example, happen in middle- and low-income countries (World Health Organization 2015).

HIV and TB are an especially lethal combination in resource-poor areas. In Zambia, where Maureen Nkhoma lives, the incidence of TB rose dramatically in the 1980s, and in conjunction with the emergence and growing presence of HIV.⁵ Debt to external lenders along with international policy reforms were gutting Zambia's healthcare system during the 1980s and 1990s, leaving the country unequipped to deal with the dual epidemic. TB medications ran short in Zambia during the latter part of the twentieth century. Antiretroviral therapy (ART) for HIV was unavailable to most people in the country until 2004, the year that ART was rolled out in government health clinics. People with HIV died ugly deaths, and these deaths were often the result of TB.⁶

Maureen's family was preparing for Maureen's funeral, her aging mother told me, when "everything changed." After months of undiagnosed illness, Maureen was hospitalized and tested positive for HIV. She was diagnosed with TB two weeks later, after her condition continued to decline while in the hospital.⁷ In previous years, both diagnoses would have affirmed the family's concerns that Maureen would soon die. However, by 2007, the year that Maureen was diagnosed, medications for TB and HIV were widely available in Zambia at no cost to patients.

The new availability of no-cost medications for HIV and TB was not unique to Zambia. At the turn of the twenty-first century, significant global shifts in treatment policies, global markets, and funding streams were reconfiguring access to treatment around the world.⁸ These shifts offered Maureen, like so many of her contemporaries in Zambia and elsewhere, hope for a "second chance at life" once diagnosed.⁹ Second chances, though, are not a given in this new treatment-focused environment. As anthropologist Susan Reynolds Whyte has observed in the context of the rollout of ART in Uganda: "To realize the second chances, care must be given and taken continuously" (2014a, 2).

On the day that I visited Maureen and her children, Maureen did not yet know if she would have a second chance at life. She labored to reach the room where I waited with Loveness and Bwalya. When she finally arrived, she sank in exhaustion into the cushions of an overstuffed chair. After catching her breath, she glanced approvingly around the room. The walls were crumbling and couches threadbare. Yet, the concrete floor was swept and polished in a deep red. I noticed with embarrassment that the only traces of dirt were in the shapes of my own shoe prints. The tables were dust-free. Decorative cloths were freshly laundered and carefully draped over each couch cushion. I followed Maureen's gaze as she inspected each item in the room in turn, and then she turned to address me.

Connecting the condition of the room to the state of her care, Maureen said: "The children are taking good care of me. People would think it is the elders who

INTRODUCTION

3

are helping and cleaning the place, but it is the children.” Maureen drew attention to the quality of her children’s care. The children were not just taking care; they were taking good care of her. Their care was so good that people would think it was “the elders.” The elders—her mother, brother, and other adult relatives—were not able to take such good care. They, too, had their own care needs and demands on their time. Months later, after Maureen completed more than eight months of TB treatment, she observed that her recovery from TB—her second chance at life, to use Whyte’s phrase—was due, in large part, to her children’s care.

A taken-for-granted assumption has shaped much global health research and policy work on infectious diseases around the world. This assumption is that adults, young or old, give care to the sick—an assumption that dismisses the many children who also give care to the sick. For example, millions of children in sub-Saharan Africa are estimated to provide some level of care as a result of the HIV epidemic.¹⁰ More thoroughly discussed in the context of the HIV epidemic is the crisis of care that HIV has created for the children after their guardians have died.¹¹ The availability of treatment for HIV has helped raise life expectancy in heavily HIV-affected areas from what it had been before treatment. As a result, many children in sub-Saharan Africa and beyond now live with adults who benefit tremendously from medications, and yet face recurrent care needs.

Children as Caregivers takes the transition to and continued pursuit of universal treatment for TB and HIV as a critical moment in which to examine care between children and their ill family members. This book is set in a particular place, a poor urban settlement in Zambia, which has one of the highest rates of infectious disease in the country. The stories I tell in the following pages took place at a time (2005 to 2008, with follow-up research in 2014) when the delivery of health services to people with TB and HIV were changing many things about what it meant to receive such diagnoses. Yet residents were, and still are, grappling with the devastation the diseases wrought in their settlement during prior decades. Even with medications, deaths from TB and other HIV-related causes continue.

At its heart, *Children as Caregivers* is about how intergenerational care happens when infectious disease becomes woven into structures, relationships, and the rhythms of day-to-day life.¹² The argument I present throughout this book is that a focus on children’s care for and by sick adults offers much-needed insight into global health problems and programs. This is no small argument; billions of dollars each year are invested in global health programming.¹³ My argument has three interrelated themes. First, global processes, policies, and programs do not just affect children. They are also transformed by the intimate and everyday acts that occur between children and their family members.¹⁴ In this respect, a study of care between children and their sick guardians can offer needed perspectives

on larger political economic shifts, among these the increasing urbanization and feminization of poverty, brutal cuts to governmental healthcare spending, the uptick in magic-bullet approaches targeting specific diseases and groups, and the significant presence of non-state actors in the provision of services in middle- and low-income countries.

Second, a focus on relations between young children and ill persons offers important insight into the forms that care and sociality are taking within settings affected by high rates of infectious disease. Specifically, within recent infectious disease epidemics and outbreaks, target group categories such as orphan and vulnerable child, street child, and child-headed household have become the focus of media attention, humanitarian work, and academic research. By highlighting children's loss and isolation, such categories suggest that abandonment and isolation are what script children's lives. A focus on children's everyday interactions and relationships not only contextualizes their lives, but also honors the dependencies and interdependencies they cultivate and the meanings of such dependencies to their own and other peoples' lives.

Finally, children shape and are affected by global health, humanitarian, and medical programs, even when such programs are designed to exclude children. The global systems of public health and biomedicine are always contextualized pursuits, as Julie Livingston has shown in her study of cancer treatment in Botswana. Everywhere in the world, Livingston has reminded us, "doctors, patients, nurses, and relatives tailor biomedical knowledge and practices to suit their specific situations" (2012, 6). Part of seeing children as social actors is acknowledging that they, too, tailor global health, humanitarian, and biomedical systems of knowledge and practice to their particular circumstances, and as a means to make life more livable for themselves and other people. Understanding how they do so, I argue, is central to making policies accountable to children's circumstances and more relevant to their daily lives as well as the lives of their family and community members.

I have chosen to focus on TB, a disease that is overshadowed by HIV in social science research on children in sub-Saharan Africa. A study of TB in sub-Saharan Africa is in one sense a study of HIV, but from a different angle. While many illnesses and other ailments are viewed as signs of HIV's presence, TB strongly indexes HIV, even in the absence of a positive test result. With the increasing presence of ART, a biomedical TB diagnosis has also become the time when many people receive HIV diagnoses because of policies of direct HIV testing after TB diagnosis.

A study of TB offers much more than a lens onto the HIV epidemic. TB is the target of one of the farthest-reaching standardized global health interventions: the World Health Organization's (WHO) directly observed treatment, short-course (DOTS) strategy. DOTS has increased the availability of no-cost, outpatient TB

INTRODUCTION

5

treatment around the world and introduced a range of practices for monitoring treatment. Zambia reached 100 percent “DOTS coverage” in 2003 when all TB treatment programs run out of government health centers were based on the DOTS principles of testing, treatment, medication, observation, and reporting. The country’s embrace of DOTS has precipitated a number of public-private partnerships aimed at treating the disease within the DOTS model.

Treatment coverage does not necessarily mean that medicines are accessible, nor should it imply that treatment adherence is an uncomplicated process of supplying drugs to the people who need them. As anthropologist Ian Harper (2006) has observed in Nepal, the DOTS protocol can feed into existing social hierarchies and be counterproductive to patient support. In this respect, attention to age and intergenerational relations offers a needed, and yet missing, perspective on DOTS in particular and standardized disease treatment protocols in general. A focus on children is especially compelling because TB has long been considered an “adult’s disease,” not only because of its association with HIV in sub-Saharan Africa, but also because of the difficulties in diagnosing TB in children. Children under fifteen years old account for only about 6 percent of the global burden.¹⁵ Further distancing children, DOTS-based programs worldwide view caregivers as integral to TB treatment success, but these caregivers are assumed to be adults—an assumption that this book will soon dispel.

CHILDREN AND THE PRACTICE OF BEING CLOSER

Throughout my research, I asked many questions and received many answers, but I have returned, in my thinking, to one particular question and its most common answer. I asked children living with adults who had TB: “How has this illness in your home changed your life?”¹⁶ I posed a similar question to the adults with whom the children lived: “How has TB changed your child’s life?” And child after child, adult after adult, offered me a similar response. The children made statements such as “I always wanted to be close to [my sick relative], now I want to *be closer*.” The children’s parents, grandparents, and other sick guardians said: “The children always wanted to be close to me, but now they want to *be closer*.” At first I did not know what to make of these answers. “Being closer” seemed like a vague answer to my equally vague questions. Yet, I found the consistent references to proximity difficult to dismiss because of the infectiousness of TB. They were especially difficult to ignore when I witnessed children’s attempts to stay close to relatives and the amount of effort entailed in sustaining such physical and emotional proximity.

The references people made to children’s proximity hint at one of the central paradoxes of infectious disease: transmissibility creates conditions in which proximity is desired and necessary, and also feared. This is particularly true in

settings where institutional and economic resources to treat infectious diseases are scarce. Relationships in such context are everything. Proximity can mark the quality of a relationship and serve as a critical strategy for social and biological survival, especially for people who are the most dependent upon others for their well-being. Let me offer an example to make this more concrete. On the day of Elesia's TB diagnosis, I sat in the house where she and her children, Abby and Chiko, were temporarily staying. Elesia was on the couch, next to several family members. Family members were discussing who in the family should care for Elesia and where they should send her children. Elesia stifled a series of coughs as she and her family worried about her six- and ten-year-old daughters' susceptibility to infection. Based on clinical advice about the infectiousness of TB and the rigorous eight-month treatment program, some family members suggested that it was best to separate Elesia from her children.

Over the course of two weeks, Elesia's sister gathered resources to move Elesia to her house on the other side of the city. Elesia's brother moved his wife and children to another relative's house to make room for Abby and Chiko in the house where he lived. Meanwhile, Abby and Chiko resisted the separation from their mother. They did so in specific verbal and nonverbal ways. They stayed closer to the house than they had before the family debate. They showed their ability to respond to their mother's physical needs, and they kept her company, prayed with her, and encouraged her to get better. They reflected on their futures without their mother and on the possibility of their mother's death while outside of their watchful care. There was no one who had more at stake in their mother's recovery than they did and this, they believed, made them most suitable to monitor her treatment and care. Elesia also wished to avoid the separation. However, by the time of the move, she was so sick that she had little energy to resist her family members' efforts.

When children spoke of wanting to be closer to specific persons, they were directing attention toward their relationships and dependencies as well as their strategies for cultivating interdependence. Their attempts to stay close evoked many of the ways in which anthropologists have studied care: as a form of work, as a sentiment and affective state, as an obligation and type of exchange, as an engagement with biomedicine, and as a process structured by social and economic inequality (Buch 2015, 279). Their care-based efforts to sustain relationships did not always work, as in Abby and Chiko's case, but children tried nonetheless, sometimes going to great extremes and, including, in the case of one young boy, running away from two different households in which he was placed.

Throughout this book, I examine the shared vulnerabilities of children and ill persons. I look most closely at how children attempted to minimize these vulnerabilities through cultivating relationships with specific ill guardians through acts of care. My intention is not to valorize this relationship between children

INTRODUCTION

7

and their sick relatives or downplay age-related inequalities and the scarcities that the children faced. Rather, I see the notion of “wanting to be closer” as a heuristic for examining children’s social action. Since the 1970s, researchers in childhood studies have argued forcefully that we cannot understand children’s lives without paying attention to children’s own perspectives on their lives.¹⁷ Their argument—an argument for which there is much proof—is that children interpret the world around them and act in ways that reproduce and also change this world. The attempt to remain close to particular ill persons, thus, represents one way in which children experienced and responded to illness and uncertainty, and also drew on the resources they had at hand—including the resources of biomedicine and the globalizing discourses of childhood—to craft particular approaches to care for themselves and other persons, in an attempt to maintain a sense of normality and retain hope for their futures.

How we define children’s care for sick adults matters. Because children’s caregiving was largely ignored by social scientists until rather recently, much research on children as caregivers has focused on the work of care, emphasizing children’s domestic and nursing activities, their cooking, cleaning, nursing, bathing, feeding, and more. This positions children’s care within the much larger body of research on children’s domestic work,¹⁸ which has made important contributions to our understandings of children’s roles in economic life and shown that children’s work can be quite significant in particular times and places.¹⁹ A fundamental premise in childhood studies is that childhood is not a fixed life stage.²⁰ The cross-cultural variability in children’s work exemplifies this premise. While there are universals shared by all young children around the world, such as small physical size and early dependence on persons older than them to meet their basic needs, there is much more variation in what children can and are expected to do. Furthermore, larger-scale processes—capitalism, urbanization, globalization, and, as I argue in this book, the changing apparatuses of global health—factor into what children do and also into ideologies about children’s care work as normative and necessary, or out of place.

There is a tendency when examining children’s work to want to identify it as good or bad, harmful or helpful. Children who carry out care work may be, in some cases, more vulnerable than children who do not, but this is not always the case. To draw a line between the acceptable and the harmful with regard to children’s domestic work, international aid organizations and policy makers have relied on particular indicators, including age and amount of time spent engaged in domestic tasks. As anthropologist Rachel Bray (2003) has argued, such measures and, in fact, the international preoccupation with children’s work above other aspects of their lives, can erase the contexts within which children’s work is situated, thus hiding more than they reveal about children’s insecurities and needs. Bray has offered the example of a young girl in South Africa. Responding

to a survey on child labor, the girl gave answers that appeared to show that she did not engage in much domestic work at all. Her answers seemed like positive indicators of her well-being. However, when Bray went to the girl's home, she noticed that there were few chores for the girl to carry out because the household had almost nothing to clean or cook. In such a setting, Bray observed, a child's involvement in domestic work might actually be a positive gauge of social and economic security. In the example from my own study, Abby's and Chiko's inability to perform care work for their mother did not mitigate their vulnerability. Instead, it threatened their ability to influence the course of their mother's illness and, in their interpretation of events, their relationship with their mother and sense of security in the present and future. The point is that standardized measures only tell a partial, often decontextualized story, and this story does not always match children's experiences or their understandings.

Because children's contributions in households are highly variable, context specific, and do not map neatly onto age-based categories, the extent and nature of children's domestic work is hard to grasp. As anthropologist Pamela Reynolds (1991) has shown in her study of children's work in subsistence agriculture in Zimbabwe, one of Zambia's neighboring countries, anthropological research techniques are critical to understanding children's work in all of its complexity. Children's work, she suggested, can be at the same time obscured, not considered work, and also highly valued and valuable. It can also vary through time with the changing needs and capacities of households and children, making it hard to understand children's contributions at just one point in time. By following particular children through time, Reynolds was able to identify that children provided adjustable labor for households, filling in where needed and assisting other household members, typically women, when they were overburdened. Though Reynolds does not address illness, her points hold especially true in care for sick persons. The notion of being closer to the sick is far less vague if we think about care needs as constantly changing and if we consider staying close to an ill person as one way in which children remain knowledgeable of and able to do something about such health needs and changes.

Children's attempts to become even closer to specific guardians during illness were about much more than providing adjustable labor. As the example of Abby and Chiko demonstrates, the notion that children wanted to be closer also hints at another form of work—the work involved in retaining intergenerational and kin relationships. Anthropologist Fiona Ross (2010) has observed that the work of retaining social relationships is unending and frequently becomes most evident during crises, such as illness and death, when relationships threaten to dissolve and when social isolation and rejection become real possibilities. She has written about this work in terms of the temporality that illness and dying impose on ill persons: "Illness, death and talk of illness and dying insert new forms of

INTRODUCTION

9

time into daily routines and everyday life. Caring is pierced by waiting; plans are hollowed by uncertainties; time becomes rigid and drifts simultaneously. It is within and across these qualities of time and not solely those of chronology, seasonality and genealogy that social worlds are crafted and refashioned. The return to life involves . . . the crafting of relationships so that they again take a recognisable and socially sanctioned form” (2010, 198). While Ross was referring most specifically to adults who were ill, her observations are suggestive of the temporality that a guardian’s illness might impose on children as well. As I will show throughout this book, children linked the survival of specific relatives to their own future prospects. Retaining closeness in proximity and affect served as a strategy to shape not only the social and biological survival of sick guardians, but also the social and biological survival of the child. This strategy was context specific and pulled from a range of discourses about proper relationships between children and adults and the types of relationships children need to attain livable futures.

Children’s attention to their proximity to sick guardians highlights the broader inequalities that the children faced in their daily lives, as well as their attempts to do something about such inequalities. Anthropologist Frederick Klaitz (2010) has offered insight on this point in his research in Botswana on how people were attempting to sustain relationships in the midst of the HIV epidemic. He suggested that broader economic inequalities in wage labor and access to resources had reinforced people’s tendencies to assess the state of their relationships through the places in which they gave and received care. In Klaitz’s words, Botswana “have tended to experience social inequalities as aspects of gendered and generational relationships within domestic or housed spaces . . . As a result, they commonly take steps to reshape their own and other people’s manner of imagining such relationships” (2010, 18). They did so through activities that occurred in households on a daily basis, such as bathing, nursing, drinking, visiting, and praying. While Klaitz’s observations were based primarily on women who were members of an Apostolic congregation, and not children, they resonate with the ways in which the children in my study worked to refashion their identities and relationships through activities that occurred between themselves and their sick guardians, in homes and on a daily basis.

Close attention to the place of care reveals both the inequalities children face and need to manage and also the increasingly global facets of childhood. Children received a range of messages about their schooling, rights, vulnerability, and work that they incorporated into their understandings of their situations and the care they gave and received (Skovdal and Ogutu 2009). These globalizing discourses of childhood, as I will argue throughout this book, cannot be measured against local discourses of childhood. They are localized in children’s everyday lives. For example, the children in my study used categories, such as the

category of the orphan, to interpret the social isolation that might occur if they were separated from a particular guardian. They understood their care work in particular places and for specific adults as a means of securing an education (and, thus, a future). They also construed activities carried out for the “wrong” adult as something that could inhibit their schooling. Abby, the girl I mentioned earlier who wished to stay with her mother to give her care, had been out of school since her mother became sick, but she saw her chances of returning to school as higher if she remained in her mother’s care, rather than in her uncle’s. In her mother’s household, she was a caring daughter who responded to the needs of her mother. The potential for reciprocity remained there, despite her mother’s debility. In her uncle’s house, she saw herself as a domestic worker whose actions and attempts to develop a relationship and also receive resources went unreciprocated.

Research on children’s experiences of caregiving frequently embraces dichotomous thinking: assessing children’s care as negative or positive; seeing childhoods as local or global; and positioning children as caregivers or care receivers. This framework, I argue, limits our understandings of children’s care. *Children as Caregivers* examines children’s care in all of its complexity and “as many things all at the same time” (Orellana 2009, 118).²¹ In the following chapters, I will show children’s care for sick persons as a process of commensurability in which some children and ill adults forged—or attempted to forge—a common vulnerability, one in which boundaries between healthy and sick, adult and child became blurred.²² It was this common vulnerability that the children and their ill guardians were referencing when they spoke of wanting and needing to remain close to one another.

GETTING CLOSER

I wrote this book as an anthropologist who has worked for a number of years in the delivery of public health in Zambia, both prior to and during the research and writing of this book. I arrived in Zambia, first, as a Peace Corps volunteer and have since worked on a number of health assessments and interventions, with (or contracted by) the World Food Programme, UNICEF, and the Zambia AIDS Related Tuberculosis Project. Throughout this time, which spanned from 1999, when I joined Peace Corps, to my most recent anthropological fieldwork in 2014, I have seen how children’s perspectives can become sidelined in the delivery of health services, even when services are aimed at children. I have also seen a growing emphasis on children’s participation in public health interventions and community development, and I have struggled with my colleagues who work in the delivery of public health to identify ways to make children’s participation in such processes meaningful.²³

INTRODUCTION

11

My interest in studying children's experiences with TB treatment and care grew out of my affiliation with a Lusaka-based nongovernmental organization, the Zambia AIDS Related TB Project, or ZAMBART.²⁴ Before beginning my own study of children's caregiving, I assisted ZAMBART staff with a project they were carrying out with schoolchildren. The project involved asking schoolchildren in heavily TB-affected settings to disseminate information about TB and HIV to their family members, with the hope that children might compel family members to get tested. It was the first TB project to involve children as health promoters in their communities, which made the project uniquely situated to answer a question that had never been asked: Do children have a role to play in TB prevention programming?²⁵ The children were already playing roles in their households, well beyond what the project envisioned. However, what exactly the children were doing in their households was hard to ascertain from their responses. We were missing the ethnographic details of children's daily lives with TB, and this raised further questions for me about what current and past public health efforts were also missing about the household management of the disease. This was a methodological problem that we could not solve within the confines of that project and certainly not within the many other studies of TB in Zambia and elsewhere that excluded children.

Much of what researchers know about people's experiences with TB comes from interview and survey-based studies, frequently carried out retrospectively and outside of the usual place of care, the home.²⁶ Such studies have identified important aspects of TB diagnosis and treatment, and yet they fail to adequately describe the range of social relationships that shape therapy management.²⁷ Instead, they hint at factors such as access to clinical resources, shortage of medications, and feelings of wellness that can only leave us to imagine the daily lives of TB sufferers and their household members. They frequently ignore children entirely. In the previous section, I demonstrated how statements about children's desires to stay close to ill persons helped me understand children's experiences and strategies in households affected by illness. The notion of being closer—or getting close—also offers an apt metaphor for ethnographic fieldwork.²⁸ Ethnographic fieldwork is about physical and social proximity: being in a particular place and close to particular people. Ethnographers live, work, or visit with people repeatedly over time. Observation and participation in daily life brings researchers closer to people's daily lives and relationships than other social science and community-based methodologies, which tend to rely on a limited number of interactions conducted outside of the usual places where people live, work, receive care, and generally carry out their daily activities.

THE RESEARCH

Because I wanted a closer perspective on daily life with TB, I chose to situate most of my research in the clinic and in houses in one particular place—George—a poor residential area that is located on the geographic and economic margins of Lusaka. Lusaka is both the capital of Zambia and the largest city in the country, with around 2 million residents in 2015, in a country of more than 15 million people. Approximately 70 percent of Lusaka's residents live on just 20 percent of the city's land, in low-income areas that encircle the city (World Bank 2002). George is one of these settlements. Through various exclusions, residents of George face high risks of infectious diseases such as TB. It has been estimated that the incidence of active TB in George is 800 in every 100,000 individuals per year (UNICEF 2008), a number nearly double the country-level incidence.

George is a favored location for international nongovernmental organizations (NGOs) and research for several reasons. The health statistics in the area provide the methodological as well as moral and practical justifications that researchers and practitioners need to show funders. There is an infrastructure for and history of health projects at the government health center in George. There is also a clear need to improve and provide services that are sorely lacking. The first time I went to George was in 2005 with my colleagues at ZAMBART, who were working with the government health center to carry out research on TB diagnostic testing and promote TB reduction initiatives.²⁹ Since that first visit, I have carried out my own fieldwork in the settlement in 2005, 2006, 2007 to 2008, and 2014, with the help of two research assistants who lived in George, Emily Banda and Olivious Moono.

The central observations in this book come from twenty-five households, each of which I visited weekly during my 2007 to 2008 fieldwork, and then again in 2014.³⁰ Of these households, seventeen households had an adult member (eight women and nine men) who had been diagnosed with active TB at the initiation of my research. Eight households were comparison households, households in which no one had been diagnosed with TB for the previous five years.³¹ Because I was interested in examining children's care, I focused most directly on children between the ages of eight and twelve years old. All twenty-five households had at least one child in this age range, for a total of thirty-eight children (twenty girls and eighteen boys). I chose this age group because children of this age are so frequently left out of global health and medical research, which has focused primarily on children under age five years and fifteen years and older. In addition to the observations, interviews, and other methods I carried out in these twenty-five households, I also administered a broader survey across two hundred households in George, carried out observations in clinical sites of TB treatment, and conducted participatory workshops with children.³²

INTRODUCTION

13

One of the many things I find so valuable about ethnographic research and writing is the ability to combine many different forms of evidence, from observations made while participating in activities of daily life to interviews, structured surveys, performances, archival materials, and much more. I find such an approach not only necessary for examining health and illness, but also critical to understanding children's experiences and perspectives. In recent years, much has been written about the best ways to include children in research, with many debates and discussions about the value of various, usually participatory techniques.³³ As will become evident throughout this book, I relied on many techniques for working with children, including observations in their households as well as more participatory approaches, such as performance, group discussions, games, and drawing. (View the children's drawings at <https://www.flickr.com/photos/childrenascaregivers/>.) Flexibility, variability, and constant reflection defined my approach to research with the children. My analysis does not privilege any one method. I draw on all of the methods I used to bring me closer to understanding children's strategies for and perspectives on care.

ETHNOGRAPHY AND ILLNESS

Ethnographic research aims to humanize people, but infectious diseases can be dehumanizing. Airborne diseases such as TB produce fears—both real and imagined, with the lines often blurring between the two. Many people over the years have asked me if I was worried that I would contract TB. I was concerned for myself and also for Emily and Olivious and the people we might expose if we became sick. We took precautions, but there were also limits.³⁴ Complete avoidance would have meant shunning the things that humanize people during illness, such as sitting quietly together or entering a bedroom to offer words of encouragement to a person who could not get out of bed. People made counter attempts to avoid transmitting TB to us. They stifled coughs and made sure that we sat near doors that were opened wider than usual to let air circulate. We paid attention to these signs and adjusted our behavior. Paying attention to proximity—and also limiting our proximity—served to build the relationships that were essential to this research.

Tuberculosis, like most serious diseases, creates crises in households that shape the research relationship, particularly during initial diagnosis and at times when symptoms are severe.³⁵ Another question that I receive frequently is: How much did I intervene in or help the households in my study? This question comes particularly from students and practitioners of medicine and public health who are new to ethnographic research and worry about the ethical implications of observing suffering. I have two responses. The first is that I did provide some assistance. Emily, Olivious, and I put people in touch with various aid programs

that gave monthly allocations of food and other resources. The three of us were frequently asked for biomedical information. Emily and Olivious gave advice based on their previous work with HIV- and TB-related programming, and we contacted doctors and TB nurses to field questions we could not answer. I sometimes gave money for transportation to health centers or the University Teaching Hospital, or for follow-up X-rays or diagnostics tests. I brought nutritious foods to households when it seemed that there was not enough food in the house. While I have no illusion that I was more than a blip in people's lives, I acknowledge that my presence shaped illness management and illness trajectories and, therefore, the research in ways that I will never fully know.

My second answer to this question addresses a broader concern, especially for people with previous experience working in or studying medicine, public health, and social work who are accustomed to interventions. I reiterate the value of slowing down in a global public health research environment that values action, speed, and efficiency.³⁶ This slowing down forefronts listening first and not assuming that we know what is best for people in different circumstances from us. In teaching interview techniques in health research, I have learned that many people unfamiliar with ethnographic interviewing are inclined toward correcting or judging people who say things that fall outside of the parameters of biomedicine, taking an expert stance that positions them as knowledge producers. This contradicts a fundamental aspect of ethnography in which informants are the knowers and the ethnographer is the person striving to know (Madison 2012). I teach that the benefits of listening and taking a nonexpert position are many. Such a position enables us to learn how things work on the ground and to get past our taken-for-granted assumptions about people's lives. It allows us to question received ways of categorizing people and problems and intervening in people's lives. That is, listening to people and seeing issues from multiple perspectives can offer more appropriate pathways for changing the delivery of healthcare and medicine (Biehl and Petryna 2013a; Pigg 2013).

OVERVIEW OF THE BOOK

Each chapter in *Children as Caregivers* engages children's perspectives on their circumstances, while addressing a different aspect of the children's daily lives. The chapters progressively build to develop a fuller understanding of the children's caregiving within a setting heavily affected by infectious disease. The first chapter, "Growing Up in George," situates the reader in the urban residential area where the children lived, describing the many factors, including global restructuring and a history of international development agendas, that have shaped the landscape and children's life chances. The children's perspectives on this

CHAPTER 2



Residence and Relationships

I caught up to Maureen Nkhoma on the dirt road that ran in front of her house. During my initial visits with Maureen, she was so debilitated that she could barely stand on her own. Five months later, she was well enough to go to the kajima several times a day to draw water. She balanced a yellow jerry can filled with water on her head. Her neck muscles tensed as the container swayed when she stopped to greet me. Slapping my hand and holding it in hers, we greeted each other:

MAUREEN: *Mwauka bwanji, Jeanie?* [How did you wake?]

JEAN: *Bwino. Mwauka banji, Amake Loveness?* [I woke well. How did you wake, mother of Loveness?]

I asked how each of her children awoke that morning and how her mother, who was visiting from the family's farm, awoke. Everyone "woke well," except for her mother, whose legs were aching after the long bus ride to George. Maureen's eyes lit up as we turned to questions about my household:

MAUREEN: *Bauka bwanji ku nyumba?* [How did everyone awake at your house?]

JEAN: *Bauka bwino.* [They woke well.]

MAUREEN: *Bauka bwanji mpando?* [How did your chairs wake?]

JEAN: *Bauka bwino.* [They woke well.]

MAUREEN: *Na tebulo?* [And your table?]

This was an old joke between us, and we laughed every time. Maureen's jokes about my furniture started months before when she realized that I lived alone. My household set-up surprised her, and she advised me to keep [*kusunga*] a child. Maureen explained it this way: I spent long days in George and so I needed

someone to clean the house, do my laundry, and make my meals. A child could help me with this work and forestall the loneliness that she knew I felt during the evenings and on Sundays, when I was not in George. Her references to my chairs, table, and material wealth were pointed. They identified the absence of people in my home and indexed my financial capacity to keep a child.

On this particular day, Maureen jokingly suggested that I keep Loveness, her eight-year-old daughter. Loveness, she said, listened well to adults, and she knew how to cook, clean the house, and wash dishes and clothes. Proof of her capabilities was in Maureen's house, which Loveness had kept clean during the worst of Maureen's illness. She talked about the things I needed to consider when keeping Loveness. Maureen said that I would, of course, find a school for Loveness near where I lived, and I would pay for all of Loveness's other needs. Maureen's jesting taught me two key facts of life: the resources and opportunities children are afforded depend on relationships, and children make day-to-day life meaningful and possible through their affective and practical actions. That is, interdependence characterized relations between adults and children.

I played along and emphasized that Loveness would refuse me (*akana maningi!*), to which Maureen laughed and replied that Loveness could not refuse the move; she was fond of me, and I had become family over the past few months of visiting them. By framing me as family, Maureen was giving me a lesson on moral expectations that family members help one another.¹ She subtly criticized my isolation and stinginess. She more openly criticized her own family members, claiming that Loveness had not received a thing from her relatives, not even an invitation to visit their house. It was unlikely that Maureen would have let Loveness move, even if family members had offered. She proved this point when her brother wanted her son to move in with him during a time when Maureen was still very ill. She asked: "Am I not his mother? He should stay with his mother." She followed this up by telling me how much her son provided her with care, something that she had also emphasized when we first met, as I described in the introduction.

At the time I write this book, observers of the HIV epidemic in sub-Saharan Africa, including policy makers, development practitioners, researchers, and the media, have expressed grave concerns about the capacities of families to care for children. It is often repeated that HIV has caused a crisis of care, in which there are not enough adult family members capable of providing care to children as there had been in the past. Such strains are deeply felt by people in George, as Maureen suggested when she criticized family members for not inviting Loveness to their house and, conversely, when she expressed resentment when family members invited her son to stay with them.

In this chapter, I examine this crisis of care by viewing children as actors in family networks. This approach offers a different analytic for understanding the

crisis of care than is typically used in research and reports to assess children's needs. For example, typically, children's needs have been examined in terms of discrete outcomes, such as nutrition, school enrollment, and unpaid labor. These measures and others are correlated with whether children's biological parents are living or dead (often referred to as orphan status), the types of households in which children reside (grandparent-headed, woman-headed, child-headed), and if children are living outside of family care, in institutions or on the streets. Such measures acknowledge the importance of family care, but they cannot capture the dynamism of relationships, the mobility of children's lives, and the roles children play in families. My overarching goal in this chapter is to fill in gaps in our understandings of and approaches to family care, both of which drive humanitarian initiatives and policy prescriptions aimed at children. To achieve this goal, I examine how children and their adult family members thought about children's kin relationships and their roles in families. I pay careful attention to children's attempts to achieve good care for themselves within exceptionally difficult circumstances and within newer global discourses of childhood.

ECONOMIC LIFE IN GEORGE

Material scarcity in George was a fact of life that structured the children's roles and relationships in and across households as well as their access to resources. Unemployment in formal sector work has continued to be high in Lusaka, especially in the low-cost settlements where many of Lusaka's poorest residents live. Only a fortunate few adults in my study—mostly men—found formal sector jobs in areas such as meat and dairy packing, road construction, and contract carpentry. These jobs were low paying with long hours. One of the highest earners made about US\$80 a month as a security guard in Lusaka's industrial area, several kilometers outside of George. He worked every day of the week, with two days off per month. His job provided more security than most, though ultimately, his sickness undermined even this security. He was fired when his absences ran over his company's allotted sick leave.

While men struggled to find and keep formal sector work, women in George faced even greater challenges in the labor market because of their lower literacy rates than men, the preferential treatment men received from employers, and social expectations that women would remain close to home as caregivers and housewives (Schlyter 2009). Only one woman in my study worked a formal sector job. She attained her job through a male relative who knew the employer and advocated on her behalf. Most women made their living through cobbling together piecework and other informal labor activities, such as petty trading. They broke stones, worked with NGOs on the improvement projects that entered into the settlement, and sold food, goods, or services (such as hair plaiting or

tailoring). Their work was time consuming and could be exhausting. One woman I knew woke at three o'clock every morning to walk several kilometers to a rural area where she bought vegetables to sell in George. She sat at her market stand during the day, remaining there until she sold the last of her vegetables at dusk. Another woman spent her days in Lusaka's town center carrying a basket of bananas on her head to sell to hungry passersby. Many women bought or made things to sell from their houses, such as fritters, grilled meats, frozen drinking water, and "ice block" (flavored ice in a bag).

The ways in which children participated in economic life were indicative of both the difficulties involved in making a living and the gendered inequalities in the labor market. In several group discussions, I asked the children to tell me their roles in economic life, using the question: "Do children provide food for the house?" In George, the phrase "providing food for the house" referred generally to a two-part process in which a person had money or other resources and shared these resources with other members of the household.² This question took into account that households, as feminist anthropologists have shown, can be sites of competition and differentiation, in which members compete over and negotiate access to resources, rights, and obligations (Moore 1994). Husbands and wives in Lusaka have not necessarily shared their earnings, with women frequently not knowing how much their husbands made (Hansen 1997). Further, women and their kin have borne primary responsibility for feeding and clothing children and ensuring that they attended school in ways that fathers and fathers' kin have not. Conceptualizing households as homogenous and discrete units when they are in actual fact differentiated and dynamic can conceal the complexity of relations among household members as well as among members of different households. Just as cultural and economic processes frame relations between men and women, they also frame relations between adults and children, particularly women and children.

My question to the children about whether and how they provided food for the household was aimed at understanding not just their contributions but how they envisioned themselves economically and within gendered and generational relations in the households:

JEAN: Do children provide food for the house?

GIFT: *Awe!* [No!] They just sit.

KATHRYN: *Awe!* They just sit.

FAUSTINA: *Awe!* They just eat.

MULENGA: Some play.

IRENE: Children go to school.

The children did sit at home, eat, play, and go to school, but they also did much more. Take Gift for example, the eight-year-old boy who first claimed, "*Awe!*

They just sit,” in answer to my question. Gift carried out labor that, in my assessment, helped support two households in his family. Despite Gift’s small size, he was very “strong,” a term his grandmother invoked frequently in reference to his capabilities. Gift accompanied his grandmother to the family’s farm for weeks at a time to help with planting and harvesting. When he was in George, he did many other things: swept, washed dishes, went to the market to buy food for his mother and grandmother, and ran errands for his uncle, who repaired cars. He drew water from a shallow well in his parents’ yard to sell to neighbors, who paid him by the bucket. He followed his bus driver father to the bus queuing area, where he sat on a low-lying wall, conveying messages, doling out information, and directing bus traffic for his father.

Given the amount of work that Gift and other children accomplished, how do we interpret phrases such as sitting, eating, and playing? Analyses of similar statements made by youth in Lusaka and farther afield in Accra, Ghana, offer some clues. In Karen Tranberg Hansen’s (2005) research with youth in Lusaka, she has called such phrases “discursive metaphors.” Metaphors such as “sitting at home,” she argued, offer insight into how the youth felt about being stuck in the compounds without opportunities for spatial or socioeconomic mobility. Drawing on Hansen’s work, Thilde Langevang and Katherine Gough (2009) have shown how young people in Ghana used such phrasing to characterize their positions within a labor market that had largely shut them out. Their primary focus was to “not spend too much time ‘sitting at home’ and ‘eating’ as this signals unemployment, laziness and consumption rather than accumulation” (749). Daily movement around the city became a key strategy for survival and attempt to cultivate personal success through seeking out work and creating social networks.

The children in my study referred to eating, sitting, and playing from their own particular social positions, which differed from those of the youth in Hansen’s and Langevang and Gough’s studies. Most of the work that children accomplished both supported women in their activities and was directed by women. In my survey of children’s activities in households in George, the majority of the 313 children ages six to fourteen carried out domestic chores in households and other activities that helped women most directly with their social expectations and labor market work. Most of the 313 children watched younger children in the household, washed dishes, and swept the house regularly. A quarter of the children assisted with market-based activities in their households, through their help selling goods out of their houses or watching an *ntemba* [small shop or stand] set up next to the house. Fewer children—only 28 out of 313—were reported as regularly helping with market activities away from the house, in the markets or “the field” (walking around the settlement selling food that they carried in a basket).

Children also supported women during a household illness through all of the above activities as well as through the nursing care they provided. Of the 313 children counted by the survey, 262 were said to collect medicine to give to the sick members of their households. However, while the children acknowledged that they did these activities, they did not necessarily consider them as their own work. Let me illustrate this point through ten-year-old Chipo's response to a question I asked in one of the children's workshops. I asked the children a question about who in a house takes care of a sick person. My intention was to disentangle children's ideas about their roles in caregiving activities. Ten-year-old Chipo, who lived in one of the comparison households (without a TB patient at the time of the study), proudly announced: "Mary's mother takes care of the sick." Chipo lived in a large household with her mother, father, and a number of older siblings and their children. Mary was Chipo's niece, who was two years older than Chipo. When I asked Chipo what Mary's mother did to give care, Chipo gave this hypothetical response: "She makes Mary help the sick by giving him the medicine, washing clothes, cleaning in the home and washing plates in the home and sweeping the home."

Children supported their households through domestic work, helped with income-generating endeavors, and cared for the sick, but their opportunities to accumulate resources for themselves or their households were limited. Only 16 out of 313 children in my survey were reported to carry out piecework, such as crushing stones or other odd jobs, on a consistent basis. This low number was due in part to the actual availability of work in George; competition for even the lowest-paying piecework was great. There was also significant shame in George associated with children's performance of labor, which many children and adults perceived as inappropriate for young children to perform year round or for the household's benefit, though they made exceptions when such work occurred between school terms and was aimed at raising money for school expenses.

Children relied on adults to meet their basic needs, and this dependence came with expectations that they would contribute to household labor. Such expectations included sitting at home in order to respond to the fluctuating needs of the household, which varied by household composition, season, and other events and crises, such as sickness or the birth of a new baby.³ This practice became even more evident during the illness of a household member. Rose, for example, made a drawing of her grandmother lying outside on a reed mat, with Rose sitting by her side looking at her. Monica told me that she gave her mother everything she wanted so that she could get better. Mary, as Chipo observed, responded to her mother's orders to care for the sick. By remaining close, children were best positioned to observe and then react to household needs as they arose.

The children most often expressed pride in their help around the house, but they at times actively resisted such work or the demands that they stay at home in case there was work for them to carry out. They resisted by leaving their houses

to avoid both hearing and needing to respond to adults' requests. Still, the children knew that leaving for long periods of time each day without a reason, such as going to school or running an errand, carried risks. Such absences could lead to discord between them and other members of their household, particularly women, if the child's help was needed and the children were nowhere to be seen. Recall my conversation with the children about whether they provide food for the household. Mulenga said that "some [children] play." The tone of his statement carried moral evaluation, with playing too much viewed as naughty.

Whereas Langevang and Gough (2009) found that sitting at home implied laziness and consumption for the youth in their study, the opposite was true for younger children in George. The children I knew suggested that moving around too much during the day could be a sign of laziness and consumption.⁴ The children described that some children played away from the house a lot and only returned home when they wanted to eat, which they saw as both disobedient and risky. It was risky because adults did not always share their food with children, particularly when they were annoyed with them. This annoyance frequently extended, at least in the children's viewpoints, from an adult's impression that the child was either naughty or lazy or that they consumed too many resources already. So as not to give such an impression, children limited their daily mobility and remained close to home, at least when adults were present. During illness, sitting at home—and staying close to particular relatives—acquired further significance as it enabled children to accommodate a range of needs of the ill person and of any women who might be caring for that person.

Anthropologist Tobias Hecht's (1998) work offers insight for interpreting children's domestic efforts. In a study of street children in Brazil, he contrasted two types of what he referred to as "home children" (as opposed to "street children"): children who were nurtured and children who nurtured. He suggested that, in Brazil, the children of the rich experienced nurtured childhood, while children of the poor experienced childhood through the acts they engaged in to nurture and bring resources into their households. Hecht identified this as a central difference between rich and poor childhoods in Brazil: "Whereas nurtured children are loved by virtue of *being* children, the love received by nurturing children is to a great extent a function of what they *do*" (1998, 80). In George, childhood might be considered through this framework, with children most frequently fitting Hecht's nurturing childhood description. Yet the children in my study were not wholly independent, as Hecht has suggested of nurturing children in Brazil. The children not only expected to do things for adults, but they derived value and benefits from doing them, including the affective and material inputs of adults. These may not be actions particular to childhood, but to vulnerability and marginalization in the labor market in general.⁵

My description of children's household activities suggests that we cannot conceptualize children in George as either dependent or independent. Adults, too, could not be considered independent, especially when they relied so heavily upon children's support to make their livings and maintain households. I find it much more accurate to characterize the relationship between the children and adults as one of interdependence (Bray and Brandt 2007). This interdependence, as both Maureen's jest and Gift's activities have shown, was not limited to people living within a singular household but also occurred across households. Just because children and adults relied upon one another did not mean that they had equal or reciprocal relationships. People in George—both young and old—thought deeply about their interdependencies as well as the inequalities in their relationships, particularly when deciding where children were kept and what constituted good care for children versus exploitation or suffering. This became particularly evident in decisions and debates about children's movements among the households in their extended family.

KEEPING CHILDREN IS HARD, NECESSARY, AND HELPFUL

The HIV epidemic has caused much speculation on changes to family unity and generosity due to the economic and caregiving strains that morbidity and premature deaths place on families. Early scholars of African kinship systems identified kin solidarity as a moral ideal that underpinned the relations of consanguines, kin related to one another through common blood.⁶ Anthropologist Elizabeth Colson reinforced this view with her observation that in Zambia during times of hunger, people “walk many miles to beg from kinsmen living in villages where there is some hope of finding food. These [kinsmen] may grumble, but so long as anything remains in their granaries or they have funds with which to purchase grain, they are likely to divide with their indigent relatives” (1958, 21). Kin generosity extended to the social parenting of children, and children circulated frequently among kin to spread out the costs of child rearing, receive schooling, provide work in households, and receive instruction from many relatives, and not just their biological parents.⁷

A longstanding body of research on children's circulation in Zambia and elsewhere in sub-Saharan Africa has identified that children belonged to many households. To quote Colson once more: “The majority of children by the time they have reached puberty have had the experience of adjusting themselves to life in a strange household, and a portion of their training has been received from outside the immediate family” (1958, 258). This does not mean that kin were always generous with children or that kin never exploited children's efforts, as Hansen (1990) has shown in archival research on children's labor in urban Zambia during the colonial and postcolonial eras. Rather, the ideal of kin generosity

in sharing children across households and providing for the children of kin was a principle through which people assessed the strength of their relationships, their own virtue, and the virtue of their kin.

Researchers and practitioners have questioned the effects of HIV on people's abilities to keep and care for children in the African countries most affected by HIV, and have revised their understandings of children's circulation in the context of HIV. Some researchers have identified children's circulation as a critical household and family coping strategy within the epidemic.⁸ Others have observed that children have fewer sources of support and places to go because of the caregiving and economic strains HIV places on households.⁹ In my observations, such processes were co-occurring, rather than competing, as children were staying in place and moving, and they provided relief to households in crisis while also burdening them. Take the following case of the Simwondes as an example.

Vailless Simwonde rubbed her hands down her legs as she considered what she needed for her upcoming trip to her husband's village outside of Kabwe, an old mining town just north of Lusaka. She had a singular purpose; she wanted to bring back a child who could help her with the cleaning, cooking, and laundry. The housework was too much for her now that both she and her husband were sick. His health had been deteriorating for months prior to his TB and HIV diagnoses, and the frequency with which he suffered periods of debilitation had increased. Mrs. Simwonde, too, was sick, but with diabetes and high blood pressure. Her doctor told her that these diagnoses were a result of the weight she had put on over the years and her love of salt. The medicines available in George never worked for her, and she could not afford the higher cost medications available elsewhere. She met the doctor's suggestion to lose weight with incredulity. How would it look for her to lose weight at a time when her husband was so sick? People would begin to talk about their increasing poverty. Her weight loss would further confirm people's suspicions about her husband's diagnosis and her own possible HIV seropositivity.

Mrs. Simwonde needed help in the household that only a child could offer, given the flexibility children had in their living arrangements—a flexibility that older people did not have. Her seventeen- and eleven-year-old sons were helping more since both she and Mr. Simwonde had become sick. But they could not accomplish everything that needed to get done. Her older son had school that kept him away all day. Her younger son, Mulenga, assisted with chores, but Mrs. Simwonde did not expect or want him to do everything. He still needed to focus on school if he was to support the family when he was older.

The creases in Mrs. Simwonde's face deepened as she assessed the gifts she would need to bring to the village for her visit. She had not been there for years and wanted to show goodwill to her husband's kin, and gifts would show that

they were doing well in Lusaka (even if they were not). Mrs. Simwonde was to enact the class differences typically involved in receiving children.¹⁰ Her gifts would signify the household's ability to provide for a child entrusted to their care. Guardians would not want a child to go so far away without some assurance that the child would derive benefits from the move. As Maureen suggested in her jesting, there were reciprocal expectations when a child moved in and accomplished domestic work. However, these were not always upheld, and deciding whom to trust with one's children was a difficult endeavor.

A couple weeks after she left, Mrs. Simwonde returned on a bus with twelve-year-old Ruth by her side. Though Ruth had always lived in rural villages, she navigated life in the Simwondes' household and in George with ease. She carried out household chores alongside Mrs. Simwonde and Mulenga and got along well with both. With Ruth in the house, Mrs. Simwonde had time to rest and her skin looked brighter than it had in a long time. Still, Mrs. Simwonde rejected the idea that Ruth was there only to work. She said that she considered her a daughter and began looking for schools in the area.

People took notice of the changes to the household since Ruth's arrival and, one afternoon not too long after Ruth arrived, an older man visited the household to tell Mr. Simwonde that he would like Ruth to come live with him. Mr. and Mrs. Simwonde were not prepared for such a suggestion. The man strengthened his argument by drawing on notions of matrilineal descent, or matriliney. In a matrilineal system, mothers and maternal relatives bear primary responsibility for children, and children tend to form closer affective and material ties to maternal relatives. Ruth, he pointed out, was related to Mr. Simwonde through her father's side. However, Ruth and he were related through Ruth's mother, making his relation to Ruth stronger than theirs. Descent has long been seen to structure kin obligations, and most groups in George might be considered as matrilineal, rather than patrilineal, where descent is traced through a father. The reality that Ruth came to live with the Simwonde family demonstrates that matriliney is neither inflexible nor deterministic.¹¹ However, the man's matrilineal relatedness provided compelling evidence of his greater rights to and responsibility for Ruth. Though he had not gone to Kabwe to meet with Ruth's guardians and had not sought their approval, as Mrs. Simwonde had, the idiom of matriliney proved enough to dissuade the Simwondes from disagreeing with the move, at least too outwardly. Ruth moved into the man's house, with the Simwondes expressing their annoyance in private.

Mrs. Simwonde said it was a mistake to bring Ruth from Kabwe, but she had wanted to avoid the longer, more expensive, and arduous trip to her own village in Northern Province. Now, though, she was without an alternative. Mr. Simwonde's eldest son, who lived elsewhere in Lusaka, gave her the money to go to her village. She returned with thirteen-year-old Veronica, and Veronica, like Ruth, became

part of the rhythm of day-to-day life in the household. Mrs. Simwonde began looking into school placements, again, this time for Veronica. However, shortly after Veronica arrived, Mr. Simwonde stopped working, the household's money dwindled, and Mrs. Simwonde's promise to send Veronica to school went unfulfilled.

Veronica was expected to do more household chores than the Simwondes' sons, but she was not, otherwise, treated so differently. The older son stopped his schooling during this same time because the family could no longer afford the school fees associated with secondary school, and they needed the money he earned from piecework for their more basic needs, rather than to be used for schooling. Mulenga, their youngest son, remained in a no-fee government primary school, though not without his own struggles to meet the multiple costs associated with free schooling. He convinced his oldest brother, who lived outside of George, to help him purchase a new uniform and shoes. He also persuaded me to help out, catching me when we were out of earshot of his parents to ask that I buy him pencils and notebooks. The things that put Mulenga at an advantage over Veronica in terms of going to school were his social connections outside of the household and the fact that he was already enrolled in school.

Illness and poverty were a part of the Simwondes' family life, as they were for most, if not all, families in George and many families in other areas of sub-Saharan Africa that are heavily affected by HIV. The Simwondes were not unique in their reliance on children or the difficulties they encountered in providing for children in their household. What the Simwondes' experience shows most directly is that households were constantly changing in terms of resources, health, and membership. Such flux meant that the benefits and difficulties of keeping children constantly shift. Adults and, as we will later see, children must readjust to this shifting terrain with a view to both short-term needs and longer-term goals.

KEEPING ONE'S OWN CHILDREN

Part of my survey of 200 households in George focused on where and with whom children lived. My snapshot of these households identified that children between the ages of six and fourteen lived in large households with many different relatives. The majority of children (77 percent) reportedly lived with their mothers, among other relatives. Fewer children lived with both their mothers and fathers (54 percent) or with only their fathers (4 percent). When children did not reside in households with either a mother or a father (among other kin), they most frequently lived with maternal grandparents or other maternal relatives, primarily sisters and brothers of their mother. The percentages above cannot adequately capture the variability of children's living arrangements. In Zambia, the notion of family is wide and can be extended indefinitely through categories such as

mother, father, grandparent, sister, brother, uncle, and so forth (Colson 1958). For example, a mother's sister can be a mother. Similarly, a distant cousin can be referred to as sister or brother. These data, then, may not necessarily suggest a discrete biological connection but rather a social one.

Knowing with whom children were living can provide some information on adults' responsibilities and rights to children. However, understanding where and why children move provides insight into the more active process of how people grappled with their rights and responsibilities to children. Based on my observations in George and the previous literature in Zambia, I had expected to identify a lot of circulation among the children in the surveyed households. Rather than validating my assumptions about the degree to which children circulated among households, however, respondents emphasized children's lack of movement. Many children in the surveyed households (85 percent of the 313 children) were reported to have lived with the surveyed household since birth, without ever making an unaccompanied move.¹²

Keeping one's own child—even if that child was not a biological child, but a grandchild or sibling's child—was an expectation of good guardianship. “Nowadays things are difficult,” one man told me in response to my questions about whether children had moved in or out of his house, “so you can't send your children.” He and other respondents told cautionary tales about why they could not send children to live in relatives' homes. Sending a child was seen as a last resort, something that reflected poorly on a guardian. As with the Simwondes' case, you never know how the living situation in the household might change and affect the child or other people in the household. Sending children away from home was seen as a violation of parental responsibility that reflected on the parent and could have implications for relationships between households and generations.

While keeping one's own children was seen as a marker of good guardianship, not receiving and taking care of relatives' children exposed a household's financial hardship. One man answering my survey clarified why no children had moved into his household with the statement: “I am failing to look after my children. Why should I let other people's children suffer [like my children do]?” Other respondents suggested that they would like to keep relatives' children, but the children's guardians would not send them, and the children did not want to come. When considering why children were not coming to live in her household, one woman suggested: “Maybe because we are not working. But before [my husband] stopped work, we used to live with a lot of [child] relatives.” The circulation of children has been seen as an important survival strategy for poor households in Zambia and beyond, but the responses I received suggested that keeping children was both an expense and a resource that poor people were finding harder to afford.¹³

Part of this emphasis on caring for one's own child reflected the discourses of development initiatives that are advancing Euro-American ideals of parenthood, the nuclear family, and child vulnerability.¹⁴ However, ideals about shared rights to children remained, even if they were upheld in discussions of breach when adults expressed nostalgia and regret about not being able to keep children because of poverty.¹⁵ I do not view such responses as suggestive of the increasing insularity of households. Children's lack of movement may have been, in part, an effort to keep family relationships intact by not overburdening kin or causing children to suffer. In chapter 5, I will show how such avoidances became a strategy to cultivate shared familial responsibilities and obligations to a child when her mother became sick.

Despite the survey respondents' explicit emphasis on the sedentariness of children, there was much evidence of children's circulation. Out of 313 children ages six to fourteen in the surveyed households, 47 children (15 percent) moved into the households of kin at some point in their lives. Of these children, most came to the household after the death of a parent or other guardian (23 children) or as a result of other forms of hardship, such as illness, marital problems, and food shortages, in their previous households (12 children). The remaining children moved in to help with chores or childcare (4 children), receive schooling (3 children), or just because the child or someone in the household wanted to live together (5 children).¹⁶

There were also 44 children—not counted among the 313 children currently living in the households—who had moved out of the 200 surveyed households during the two years prior to my survey. Most of these children were temporary household residents, who had come in because of crises (mostly the death of a guardian) and who left because the household was “suffering too much,” another relative wanted to live with the child, or the child wanted to leave. The responses to my survey suggest that adults continued to enact shared responsibilities for children, but that, most frequently, extreme crisis propelled such children's movements into and out of households.

Beyond the survey, my observations in Zambia over more than a decade led me to suspect that there was even more circulation than the survey revealed. Adults were going to great lengths to foster belonging for children in their households, through “forgetting” children's moves, both on the survey and in everyday conversations. I witnessed this in the households I followed for more than a year when I belatedly learned about a child's parentage or their earlier moves. My observations match those of other researchers in sub-Saharan Africa, who have noted adults' efforts to protect children through reframing children's relationships and compelling children to forget their first homes (Archambault 2010, Ross 2010). A number of factors shape adults' reasons for forgetting children's moves, among these are the global discourses advanced by humanitarian efforts

aimed at AIDS orphans, which I will discuss in more detail in the following sections and from the children's points of view.

CHILDREN'S RELATIONSHIPS IN AND ACROSS HOUSEHOLDS

The above material raises the question: How do children feel about their circulations and interpret their belonging to households when their moves were both necessary and difficult? I consider the cases of ten-year-old Abby Banda, whom I have introduced in the previous chapters, and twelve-year-old Musa Njovu to illustrate broader aspects of how children experienced and strategized where they were kept, from the points of view and the actions of the children. As both cases make evident, belonging to a household was about much more than accessing material resources or carrying out domestic work. It was about the relationships children were able to build or not within particular households, and what such relationships meant for their future life chances.

Abby

When I first met Abby, she was living in an uncle's house with her mother and siblings. Her uncle, an unmarried relative who worked as a bus conductor, spent long hours away from the house. Elesia, Abby's mother, and the children moved there one night after Abby's father had forced them out of his house. The reason he kicked them out, I was told, was that "he was tired of having a sick wife." Elesia had been sick on and off for years with what she considered recurrent meningitis. She suspected that she had meningitis again at the time they were forced from their home, though after moving into the relative's house, she found out that she had TB. As I described in the introduction, her diagnosis compelled her relatives to move Abby and her sister to the family house in a different part of George, to be cared for by another uncle, Elesia's brother. Elesia moved across Lusaka to receive care from her sister.

To understand how Abby understood her care during this time, I asked her to draw "all of the people who care for you" and "what they do to take care," an assignment I gave to all of the children. As with all of the drawings the children made, Abby and the other children produced varied depictions of their caregivers based on their past experiences and their day-to-day realities at the time of the drawing. They tailored their drawings to what they thought I might wish to see in such a drawing and also in response to the reactions they expected from other children and adults present during the exercise. In other words, the drawings were a lot like the verbal or written responses of research participants, young or old, to any interview questions; they were intersubjective (Toren 2007). These aspects do not reduce the value of children's drawings for understanding children's views on their own situations or the nature of familial care more

RESIDENCE AND RELATIONSHIPS

61

generally. Instead, they make them particularly informative for illustrating not only the decisions children make about whom they consider caregivers and the places where they might receive good care, but also their interpretations of these decisions.

Using a pencil, Abby outlined four figures standing in a row. She drew an aunt first. An uncle stood next to her aunt and next to him stood the man with whom she was living at the time of the drawing, her mother's brother.¹⁷ Last in the line was her mother. After outlining their bodies, she colored them in hues of brown, orange, yellow, and red. Her aunt was robust and wore nice clothing. She had her arm extended and a kwacha note in her hand. Her uncle also looked well off, though he held his hands by his side. Her mother's brother, with whom she lived, was very thin and wore the tattered trousers of a poor man. Her mother looked healthier than she appeared in real life, indicative of the way Abby wished her to be. Abby described each figure in turn:

HER AUNT: "My aunt bought me a uniform, socks, a bag, and shoes when I was going to school. She asked me to live with her so she could send me to school."

HER UNCLE: "This is my uncle. He told my mother that she should look for a school for me. If she finds a school, he will pay for it."

HER MOTHER'S BROTHER (*in an annoyed tone*): "He keeps me and my sister. He only buys us food. Since we arrived, he has not bought us clothes."

HER MOTHER: "My mother feeds me, buys me clothes, and cares for me."

When Abby made her drawing, she had not intended to depict her father, the man her mother had married when Abby was a baby. She added him only when Olivious mentioned that he was not pictured. She penciled him in as a stick figure and said she did not want to color him because he had a "bad heart." The entire time they lived together he either wanted her gone or treated her as hired help. He had never given her anything.

Abby's drawing and related discussion offer several clues into the ways in which the children conceptualized care. She made a distinction between "keeping" and "caring" that revealed both her expectations and resentments. The Nyanja word *kusunga* translates roughly as to keep, raise, bring up, or look after others. *Kusunga* is almost always used in reference to the activities that occur in houses. The word typically refers to children, but can also be used to describe keeping anyone who is perceived as a dependent, such as the sick, elderly, and even women. Children made distinctions between *kusunga* (to keep) and other words for care such as *kusamala* (to care or protect) and *kuthandiza* (to help). For example, someone might say: "*Andisunga ndi a Bwalya, koma sandisamala bwino* [the one who keeps me is Bwalya, but he does not take care of me properly]."¹⁸

When Abby spoke of her uncle as keeping but not caring for her, she was expressing resentment for unfulfilled expectations. I suspect that her statement was made in reference to the large burden of household work she had to accomplish at the time of the drawing and her fears about her future. At the bottom of her page of caregivers, she drew herself in three scenes. The first scene was her eating with her mother, and the second was her eating with her mother's brother. The last scene featured her eating together with her sister. Elesia's brother—pictured in the middle scene—had gone to great lengths to accommodate Abby and her sister, notably moving his own wife and children out of the house so that he could fulfill his obligations to his sister. However, at the time of the drawing, he spent most days trying to find work or carrying out piecework, making Abby carry out much of the household chores and care for her younger sister. She worried that she was gaining the identity of a domestic worker, and she expressed concerns about whether she would receive her education. These concerns came out clearly when her aunt asked Abby to move to her house so that she could send her to school. Abby was suspicious of her aunt's motives. She thought that she wanted her only for housework because she had proven herself to be a good worker in her uncle's house. She worried that her aunt might not honor her promise to send her to school.

In the children's terms, to care for a child, rather than simply keep them, means to advocate for their well-being. Abby and other children expressed that having an advocate in their household mattered. An advocate provides for the child, gathers resources from kin, and organizes a child's schooling and other needs. Advocates were people who had love for children and encouraged and taught them in ways that helped them grow to full personhood. Advocates were interested in a child's future and warded off ill-meaning kin who might wish to take the child into their households because of their ability to accomplish domestic work, without consideration of the child's needs. A child might have advocates in a couple of houses and move between them, as a couple of children showed when they linked houses together in their drawings on topics of family and also of care.

Children depended on adults for many things, including growth toward social personhood,¹⁹ and this dependence figured into how children viewed adults' dependence on them. Abby and the other children made key distinctions between activities they did for adults that they considered as "work" versus "help." These distinctions did not hang on the type or amount of work but upon the relationship between themselves and the persons for whom they carried out chores or other activities. Instead, "work" referred to domestic and other productive activities carried out for people who did not care for them materially or affectively, and did not help them grow toward adulthood. "Help" was domestic work and other productive activities that the children felt were reciprocated. The distinctions they made related to what they received from adults that helped

them in the present, but most especially into the future. These included an adult's love, loyalty, and encouragement as well as their ability to facilitate a child's schooling and access resources such as clothing and food.

Even though many people could and did serve as advocates, Abby and other children emphasized the role of the (social or biological) mother. This suggests specific concerns about what might happen to their well-being in the absence of a mother or mother-like figure. Their concerns reminded me of the adults who spoke of the need to care for one's own children. Further, as I will show in later sections, their concerns touched on ideals of parenting that were emerging within the current political economic context in Zambia and also within a humanitarian context that has emphasized the plights of orphans.

Musa

In May 2008, Musa Njovu's mother, Judy, received two calls from relatives living on the Copperbelt. The first call was from a newly married couple with well-paying jobs and a newborn baby. They wanted twelve-year-old Musa to come live with them. They asked that he help out with the baby and housework and, in return, they promised to send him to a good private school. Their decision to ask Musa, rather than other child relatives, to move to their house was most likely shaped by the extreme situation in Judy's household that would make the opportunity both attractive and difficult to decline. For months the members of Judy's household had barely made ends meet. Exacerbating the strain on the household, Judy was beginning to suffer extreme joint pain. The second call that Judy received was from her oldest daughter's husband, who also lived in the same city on the Copperbelt. Hilda, Musa's sister, was critically ill, and Hilda's husband was overwhelmed by the severity of her illness and her nursing needs. He needed Judy to help with her medical appointments and nursing care. This latter call meant that there would be no hedging about Musa's move. With Judy leaving, the household had few other choices: Musa would move in with his newly wed relatives.

"Are you excited about going to a private school?" I asked Musa the day before he left for the Copperbelt with his mother. Musa shrugged in a way that made it hard to tell if he was excited, resigned, or just thought my question was odd. After all, it did not seem that he had much choice in the matter. His mother also seemed resigned, repeating that Musa's best chance for a good education was away from George. Since Musa's older brother had died the year before from TB and other complications due to AIDS, Musa had taken on many of the household responsibilities. His mother, who was still distraught from her son's death, spent most of her time at church, receiving financial, emotional, and spiritual assistance from fellow church members. Musa's two older brothers and his younger brother were busy in school. They spent most of their days away from home.

Musa was usually the only one home during my visits, and he was often carrying out chores or studying on his own when I came.

I am still unsure about Musa's parentage. When I asked his older brother once why he did not help Musa with the household chores, he told me that Musa was the son of a deceased man who had not married Judy. At the time, this seemed like a peculiar response to an outwardly unrelated question. It was something that Judy never admitted, saying that Musa's father was the same man who fathered her older children. He too had died years ago. Musa's younger brother, Mumba, had a different father who not only acknowledged Mumba but also gave Judy money to pay for his private schooling. What seemed obvious was that Musa was unable to draw on the resources of other adults as freely as Judy's other children. When chores needed to be done, they fell on Musa, leaving a number of observers in the neighborhood commenting that Musa was "the woman of the house." I heard this phrase spoken about boys and girls (including Abby) at other times in my research when children took on the bulk of the housework. It not only pointed to the gender construction of housework in Zambia but also indexed boys' and girls' undirected (by an adult) and excessive housework as out of place.

There were two aspects of Musa's schooling and education that made him more available for housework than his brothers. He was in fourth grade at the nearby government school where he had not yet learned to read and write and his school day lasted roughly three hours. His older brothers had received private school educations when they were young and their father was still alive. Musa began his schooling after this father had already died. In the eyes of his older brothers, the facts of his illiteracy made his primary schooling less important than their secondary schooling and the all-day private schooling of his eight-year-old brother. The reality that Musa's school days were so short also meant that Musa had much more free time than anyone else in the household. In other words, a number of factors combined to make him the least materially supported member of the household. His mother worried about this and viewed the move to the Copperbelt as an opportunity for Musa to receive a good education.

With borrowed money, Judy bought Musa new clothes for the journey, and they set out on a bus together. After staying only four days in the house of the newly wed relatives, Musa ran away. He found his chance to leave when his "sister" (Musa's term) took her baby to the market in search of school shoes for Musa. She returned with shoes in hand to an empty house and frantically called Judy to let her know that Musa was missing. Judy set out along the road to the relatives' house, attempting to cover the ground that Musa might walk. When she could not find him, she went back to her sick daughter's house and waited. Just before dark, Musa showed up, surprising everyone with his ability to navigate the streets on his own. The effort he went to leave the house made the family

reconsider his move and, several days later, Judy returned to Lusaka on a bus, with both Musa and her ailing daughter by her side.

I headed straight to Musa's house when I heard he had returned to George. We sat together on a couch. I asked trivial questions about his trip: What was the house like? What did you think of their new baby? Was she a good baby? Finally, I worked up the courage to ask why he had run away. He said: "Me, I can't stay without my mother. I am used [to staying with her]. I have been staying with my mother my whole life, and suddenly I start staying with my sister. It was very difficult for me so I decided to come back." I asked what he would have done if his mother had refused to take him back or the newly wed relatives had refused to let him go. He said that he would have waited until the end of the school term, at which time he would ask to visit his mother in Lusaka and then, during his visit to Lusaka, refuse to return to the Copperbelt.

To justify his decisions to his family and me, Musa drew on child protection discourses that focus on orphans, but with a twist because his mother was still alive. Mothers, he argued, rather than other relatives, love a child enough to give them proper care. If a mother is alive, other relatives will always wonder why she has chased her child from her home. A child who is chased from home becomes just like an orphan. In Musa's account, social orphanhood could unleash a chain of negative events. Relatives and other community members would use the rejection as an excuse to neglect the child, make him work, or send him to the streets. Parental death offered a ready explanation as to why relatives should care; being cast away by a living parent did not. There was something about a child moving in the absence of parental death that indexed a larger problem with the household, the child, or the parents.

In Kristen Cheney's (2007) research in Uganda, she has shown that children draw on the language of international child rights discourses on issues such as schooling and abuse to justify the difficult decisions they make. Such discourses, she suggested, could shape children's actions and hopes as well as add to their anxieties when children were powerless to change their situations. Musa's defense of his move back to his mother's house provides yet another example of how children interpret and repurpose global discourses within the contexts of their lives. Musa made a connection between his circumstances and that of orphans, who have become a main global humanitarian category in southern Africa in the HIV era. The importance of examining how children incorporate such discourses into their understandings of kin relationships was brought home to me throughout Abby's emphasis on advocating kin and Musa's emphasis on living with his mother.



Fig. 2.1 Billboard in Kalingalinga in 2008, an example of the child protection messages that were part of the built landscape. A child's voice commands adults to protect, rather than abuse, children. The billboard does not make explicit the orphan status of the child, but the grandmother figure gives the impression that the child's parents were not present in the child's life. Photo by Nicholas Kahn-Fogel.

GLOBAL DISCOURSES OF CHILDREN'S RESIDENCE AND RELATIONSHIPS: THE ORPHAN

Since the early years of the HIV epidemic, the orphan, defined as a child who has lost one or both parents,²⁰ has been viewed as the quintessential vulnerable child.²¹ The discourses around orphans are powerful and come from diverse sources: the media, international NGOs, government officials, and school lessons, to name a few. However, despite the diversity of sources, the stories about orphans can

follow a fixed trajectory, giving the reader a feeling of *déjà vu* with each reading. South African researchers Helen Meintjes and Sonja Geise (2006) have named this trajectory the “orphan mythology.” Such accounts move implicitly—and sometimes explicitly—from notions of parental death to other risks, such as life on the street, physical or sexual abuse, excessive labor, and school dropout. For example, Meintjes and Geise described a 2003 United Nations Children’s Emergency Fund (UNICEF) report on orphans and vulnerable children in Zambia that made the claim that most street children in Lusaka were orphans. In fact, as Meintjes and Geise (2006) pointed out, 78 percent of children cited in the report actually had a living parent. As they argued, policies and programs in southern Africa have been so preoccupied with the absence of families and deaths of parents that they have ignored the presence of adults, including living parents. The orphan discourse is, thus, all about children’s residence and their family relationships, though often in absentia.

Though that 2003 UNICEF report is now quite old and there is a general sense among the humanitarian workers I know that the term orphan can be stigmatizing, such claims have continued. These claims are used to draw attention to very real poverty and suffering and, importantly, compel funders to action. Take for example the comments of Edgar Lungu—Zambia’s president as of this writing (then minister of home affairs)—in 2013, after visiting Misisi, an area of Lusaka that is similar to George in its economic marginalization and burden of HIV and TB. In a *Lusaka Voice* article on December 23, 2013, Lungu was reported to have said: “The serious consequences of HIV and AIDS and tuberculosis in Misisi have subjected many children to misery, leaving them with no hope for a better future, hence they end up on the streets . . . This in turn increases the burden on society. The number of street kids in Misisi keeps rising by the day, which also leaves the girl child with no option but to be forced into sex work for survival.” While he avoided mentioning orphans, the category was very much implied as the consequence of HIV and TB.

Children were familiar with the large media and programmatic focus on orphans, especially the particular mythology about the trajectory of children from loving, stable homes to abusive homes or to life on the street (see fig. 2.1). The pervasiveness of these discourses became especially evident during almost all of the children’s tape-recorded storytelling. I had given every child drawings to use during their recorded storytelling, including two different drawings of children seated alone. Speaking into the tape recorders, most children referred to the children in the drawings as orphans. Some related the drawings to their own experiences, and many offered accounts of what happens to orphans—“they later become street children”—or they made more general pleas for mothers to care for orphans.

Take for example the recording made by ten-year-old Sarafina, who spoke in English to convey the lesson in her recording. The children rarely spoke in

English and typically only to repeat lessons, songs, or poems learned in school. To make the recording, Sarafina went outside of her house and called on her younger neighbor to participate:

SARAFINA: If there is a child without parents, she will always stay alone. Some are always crying because a lot are beaten in the streets. We see them. Some don't even go to school. We see them playing around the compound. So we should be keeping orphans. Please we beg you mothers, we should help them in any form of help. Please, mothers, we beg you.

SARAFINA: [To her neighbor] Repeat after me. If in your area there are orphans we should be keeping them.

NEIGHBOR: If in your area there are orphans we should be keeping them.

SARAFINA: You should be buying clothes for them and food.

NEIGHBOR: You should be buying clothes for them and food.

SARAFINA: We must cooperate to help them so we beg you mothers to take care of orphans.

NEIGHBOR: We must cooperate to help them so we beg you mothers to take care of orphans.

SARAFINA: And even clothes and food are important. Please please mothers, we beg you. So we are asking our mothers to cooperate with others in helping orphans.

NEIGHBOR: And even clothes and food are important. Please please mothers, we beg you. So we are asking our mothers to cooperate with others in helping orphans.

SARAFINA: We beg you our mothers. Okay, let's get in [the house].

The focus on mothers is accentuated in Sarafina's exposition about orphans in a way that calls forth broader notions of the responsibility of mothers, which Musa also referred to and which I discuss in detail in chapter 5.

Twelve-year-old Samson also focused on women's responsibility to protect children in his response to one of the drawings. Speaking for the girl in the drawing, he said: "I need help because I don't have a mother." He continued to explain: "She gets no response from the people near her. This girl needs help like accommodation, protection, and food." In both Sarafina's and Samson's accounts, the children needed help because they did not have a social or biological mother. Still further, they suggested a failure of kinship care that necessitated a broader community response to helping children. They called for new mothers to step in, though it is unclear if such mothers do. In Samson's words, "She gets no response."

RESIDENCE AND RELATIONSHIPS

69

Abby drew on similar understandings of orphans to interpret the events in her life and express fears about her future. She began her story by saying, “Here I see the child crying.” She continued:

Her mother died. When your mother dies, even if your aunt takes very good care of you, still you will have memories of the times you spent with your mother. Death is very bad. That’s why this girl is crying. Even me, it would hurt me very badly if my mother had died when she was sick with TB. Who would take care of me? As to my side, I always pray to God to let my mother take care of me at least up to a stage where I can take care of myself and do the same to my sister and my brother. Not her dying, leaving me at this stage. No! No! Because some aunts say: “They are orphans, I will do whatever I want to do with them.” That’s what happens to orphans. They later become street kids. No one can forget his or her mother!

Her orphan narrative focused on the possibility of her own mother’s death and the fear that she might have no one to care for her. She described the process of forgetting one’s first mother as fraught because relatives might not allow such forgetting when they treat children as orphans.

Abby described a key aspect of the children’s understandings of orphanhood: orphanhood occurred through familial and social rejection: “Some aunts say: ‘They are orphans, I will do whatever I want to do with them.’ That’s what happens to orphans. They later become street kids.” Becoming an orphan related to the death of her mother, but it was more so about the loss of nurture and protection from relatives in a mother’s absence. In the previous section, Musa demonstrated that death does not have to occur for a child to become an orphan. A move away from a mother figure or person who advocated for a child could make a child an orphan. An orphan was, thus, not a static category defined by parental death, but a fluid category that children could move into and out of, depending upon their place of residence as well as the quality and social perceptions of their relationships. An orphan was a child who was cast away, uncared for, and unloved. The term marked a child who had been removed from recognizable forms of sociality in which adults—primarily women as mothers—gave care and love to children. The children tried hard to avoid orphanhood through nurturing specific relationships and remaining in or trying to get back to specific places. It is notable that, according to international standards, Abby, Musa, Samson, and Sarafina were already “orphans” given that each child had experienced the death of one biological parent. But this was not an identity that they claimed for themselves and neither did seven other children in my study whose biological mothers or fathers had died.

To emphasize the importance of the children’s statements and my points, I return to Meintjes and Geise’s critique of the 2003 UNICEF report that suggested

that most street children were orphans. They argued that if implying that most street children in Lusaka are orphans “is a decisive rhetorical strategy to bring attention to a desperate situation, we are troubled by the inflammation of the orphan mythology, for in longer terms this is a powerful and counterproductive form of othering” (2006, 411). My observations suggest that Meintjes and Geise’s longer-term projection was materializing in George. Children were resisting such othering through their residential and relationship choices and also using it to inform their fears about the future when such choices were out of their control.

“GOING ON HOLIDAY”: MOVES THAT DO NOT STIGMATIZE

Children did not react to all moves away from their usual homes with resistance and trepidation. Sometimes children desired to leave the places where they lived or that they considered home. In this section, I discuss a type of circulation that the children spoke about with anticipation and excitement. It is a form of shorter-term circulation among households in which children engage in Zambia and other parts of sub-Saharan Africa. This movement is so mundane and ostensibly happy that it has gone virtually unnoticed within concerns about children’s movements in crisis. As I will show, it is nevertheless important for understanding kinship care for children who live in areas affected by illness, death, and poverty. This practice is called “going on holiday.”²²

In Zambia, the word holiday—spoken in English—refers to school breaks. Schools have three month-long holidays—one in August before the start of the new school year, one during Christmastime, and another in April. Each break represents an opportunity for a ritualized form of movement from children’s usual homes to the homes of relatives. Far from trivial, shorter-term movements may serve as critical moments to nurture reciprocity and care between households and also make and unmake particular kinds of childhoods.

On a chilly July morning in 2008, I finished sweeping a layer of brown dust out of the demonstration building on the grounds of the government health center in George. The building was one large room—typically reserved for storage and lined with bicycles and wheelbarrows donated for health projects. That day, I was conducting one of several workshops with the children. We started the day with a group discussion, which I had developed into a game. The object of the game was simple. I had made a large foam die with pictures of different topics pasted on each side. The children took turns throwing the die and drawing questions from brown paper sacks that corresponded with the side on which they landed.

Despite the playfulness of the method, the tone of our discussion was serious. The children outlined why and when children move households and talked more deeply about the ramifications of movement. They spoke of death as one of the only times when a child would move away from her home. They used terms such

as “orphan” and “street child” and connected movements with isolation, a lack of care and love, homelessness, abuse, and exploitation.

But then the tenor of the discussion took a dramatic turn. Here’s what happened: Mulenga passed the foam die to Annie. She threw it in the air; it bounced off a bench and landed on a drawing of two houses connected by a path. This was an image many of the children had drawn during my visits to their homes to indicate connections between households. Reaching into the accompanying bag, Annie pulled out the question: “What is holiday?” Chatter filled the room. Several children jumped out of their seats.

Holiday was exciting. Speaking in Nyanja, Annie explained, “Holiday is when you tell your mother that you want to visit relatives when you close school. Maybe you get escorted by an elder sibling. And when school opens, you come back.” The type of movement Annie described was so common among the children in my study that nearly half of them were away from their homes during each school holiday. In the larger survey I conducted among 200 households, I found that 71 out of 313 children ages six to fourteen years old had gone on holiday during the last school break, roughly 23 percent of the sample.²³ While this number is less dramatic than my observations during the long-term household study, it still shows a substantial amount of movement during school holidays.

The children waited for Annie to answer the question about holiday, then they spoke over one another to describe the contours of holiday. It was hard to overlook the pride, excitement, and anticipation generated by my holiday questions (see fig. 2.2). The children described the aesthetic pleasure of holiday. Holidays were for socializing with relatives they rarely saw, listening to music, dancing, and participating in celebrations. Ten-year-old Rita captured this aesthetic pleasure in a pencil drawing she gave me upon her return from a holiday spent at her granny’s house. The drawing included relatives who came for a Christmas party. When describing the drawing to me, she pointed to a radio: They had listened to traditional Zambian music and danced, she told me.

Part of the aesthetics of holiday revolved around the food and gifts that the children received from relatives. In Zambia, as Henrietta Moore and Megan Vaughan (1994) have shown, the provision of clothing and food has long made tangible the social relations across households and contributed to the fulfillment of obligations. Children emphasized that holidays were not just times to attain clothing and other needs; they were times to attain *school* clothing and needs. As I have shown, the children strongly associated school with their current and future well-being. Even small contributions toward their schooling held tremendous symbolic value, representing a relative’s investment in their future. The children’s drawings of the people who cared for them shifted substantially after their return from holidays as the children highlighted relatives who lived across or outside of Lusaka, rather than the household members who were typically depicted in such drawings.



Fig. 2.2 Drawing of "going on holiday." Tracy shows the connection between her parents' and grandparents' houses, which became evident during her holiday visits with her grandparents. Tracy wrote, "I love my mother and father" and "I love the house," in reference to her grandparents and their house. Drawing by Tracy, age twelve.

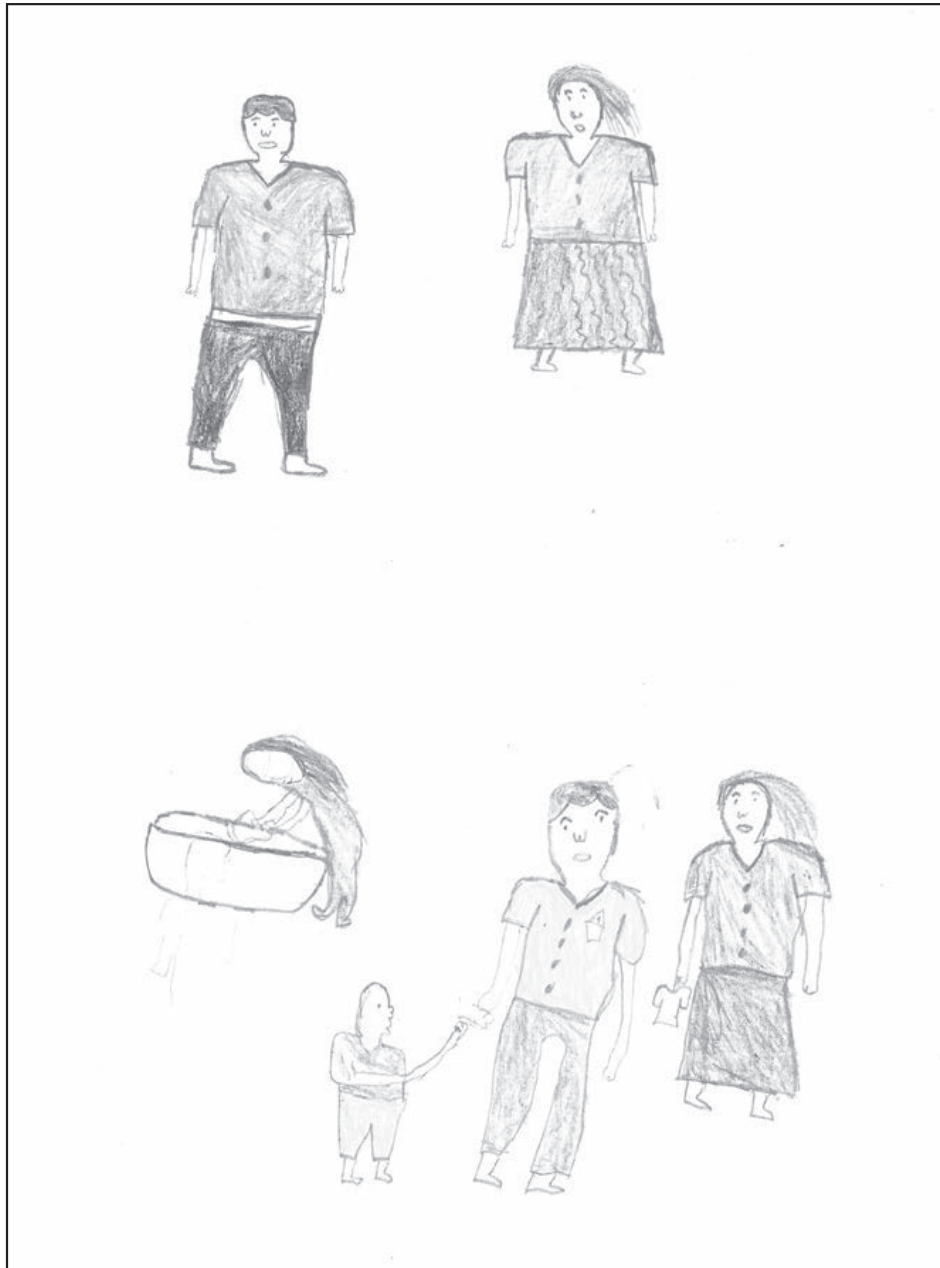


Fig. 2.3 Drawing made in response to the question: “Who takes care of you?” At the top, Stephen drew his uncle (father’s sister’s husband) and his aunt (mother’s sister), who live in different parts of Lusaka. Underneath, he drew his aunt washing clothes for him while he visited her during holiday. He also drew his uncle giving him money during a holiday visit and his uncle’s wife (his father’s sister) giving him clothes that she had bought for him. Drawing by Stephen, age eleven.

Holidays were not, however, primarily about money and financial support for children. The children expected to become embedded in the division of labor in the households. As Eness explained: “When they go for work, you sweep their house and wash plates.” The children highlighted that holidays were times to contribute to the care of grandparents, newborn babies, or sick relatives. They indicated the importance of such caring and domestic activities and took pride in their ability to contribute.

In describing the many aspects of holiday, children were referencing a particular locus of care that extends across households. It was a locus of care in which they participated and benefited and in which they played important roles. With kin spread across residential areas in Lusaka, holidays provided a time for children to nurture relationships in multiple households—their usual households and the households they visited—while remaining in school and not overwhelming kin or moving away from their preferred home. It may not be too far of a reach to suggest that such work across households and within shortened time frames contributed to the survival of some households. Regardless of their impact on survival, holidays served as attempts to forge forms of sociality that, in Patricia Henderson’s words, “looked to the future, but that were deeply imbricated with repertoires that had some continuity with the past” (2013, 14). While Henderson was writing of the maintenance of marriage rituals in South Africa’s HIV epidemic, holidays were also a sort of social occasion. By setting the children up as school-going, contributing, and cared-for children, this social occasion strongly resisted discourses about child isolation, abandonment, and family breakdown, which I described in the previous sections of this chapter.

Holiday presented a time when constructs such as kinship and home were mobilized to naturalize and also call into question social divisions, categories, and identities (Ngwane 2003). As I will show later in chapter 5, a child’s holiday movement to a relative’s house helped maintain the appearance of a normative childhood for a girl whose mother was ill. In another household, a child’s holiday movement offered a sick woman the help she needed to maintain her house and marriage.

Holidays could also naturalize differences, such as the difference between school-going children and out-of-school children, relatively well-off and poorer households, and children with nearer or farther away family members. Particular children were more desired as holiday visitors than others. This followed along various lines of difference among children and households, with some children desired for either their ability to contribute labor or their school-going status and potential for success.

As a normative form of movement, holiday offered an opening for a number of processes and practices that could not happen at other times of the year without causing social disruptions. Some parents and guardians worried about

the motives of relatives, who might wish to keep a child after the holiday ended. One woman explained to me that she did not allow her granddaughter to go on holiday because it meant she would leave school permanently. She believed that relatives would become accustomed to her granddaughter's productive contributions in their household, viewing her more like an orphan than a daughter. Her suspicions proved partially correct during a later school holiday when her granddaughter left for holiday and did not return to her household or to school. However, complicating ideas about who orchestrated that move, children in the girl's neighborhood told me that the girl had wished to move out of her grandmother's house for the longer term and stay with the relatives she was visiting. I observed other children orchestrate moves away from their households under the guise of going on holiday. Musa told me he, too, would have moved back to his mother's house during a school holiday and refused to return to the relatives' house on the Copperbelt. In this way, children considered holiday as a time to assert some control over their residential status and try out life in a different household, without damaging relationships or identities.

CONCLUSION

I began this chapter with a singular objective: to identify how children and adults were attempting to secure and enact good care for children at a time when HIV has affected many families and when households face chronic resource shortages. The fact that children as young as the children who took part in my study were not supporting themselves may seem obvious to readers. Yet this chapter should also make clear that children play an important role in the domestic economy, even when their actions are supportive of and initiated by adults (as opposed to self-guided and independent).²⁴ As I have shown, neither dependence nor independence fits what it meant to be a cared-for child in George. Instead, children and adults were dependent upon one another; they were interdependent.

In George, and likely many other places around the world, it is impossible to divorce what children do from what they receive and, yet, scholarly and policy discussions tend to separate children's doing from their receiving. Putting these two together—what children do and what they receive—can help us understand children's strategies and fears. For example, Pamela Reynolds has written about children's economic contributions and strategies in Zimbabwe. She suggested that, through both small and large activities carried out for adults, children in the rural area where she worked developed strategies to nurture specific kin relationships (1991, 77). My work has shown that children viewed such nurturing of relationships as instrumental to receiving the many things they needed in order to grow into social adulthood. Not all kin relationships were viewed the same, and children attempted to direct their productive activities toward the relationships

that mattered to them. This depended upon their residence, which offered them proximity to people who advocated for them in many ways.

When talking about children's family care in George, it was impossible to ignore children's circulations and also the lengths to which children and adults went to keep children in place. The adults I surveyed underscored changes in children's circulation due to the poverty they and their families faced. Everyone, they said, must take care of their own children. Their emphasis on children staying in place may indicate that people were ascribing to a much more rigid concept of parenthood and home than typically described in previous studies on child circulation in sub-Saharan Africa (Archambault 2010). And yet, my survey suggests that children were still moving to help and be helped by adults, and adults were upholding strong values associated with caring for kin.

The children I knew were aware that residence in certain households, with certain people, could shape their identities and future prospects. They expressed that the key to long-term well-being was the ability to remain in the care of specific people who affirmed their identities and advocated for them. Moving to or living in particular places denied them such identities and relationships, or at least raised concerns about such a denial. A notable force in Zambia has been the large amount of attention and funding set aside for orphans, children whose parent or parents have died from HIV. Orphans have remained a consistent focus of international HIV relief funding, even though variables such as poverty prove more indicative of a child's needs than whether or not their parents are alive.²⁵ Children drew on the particular orphan mythology that has arisen to draw attention to children's needs—the mythology that orphanhood is equated to life on the streets, abuse, school dropout, and neglect. In their experiences, an orphan was not, however, defined by death of a biological parent, but by rejection. The mythology presented a grim picture of what it meant for their life chances when they moved away from adults who might advocate for them, particularly mothers, or when such moves appeared imminent because of illness.

Not all moves away from their households were so negatively evaluated. Holiday visits were less permanent and served as a creative attempt to nurture kin relations, reinforce particular valued identities, and exchange labor and resources in ways that current contexts and discourses make difficult. Holidays drew on conventional understandings of the circulation of children, but without the strains on households or stigma of circulations in which crisis is directly implicated. In its idealized form, going on holiday reinforced broader conceptualizations of the meaning of a good childhood in George. This included an emphasis on schooling, children's domestic contributions, and adult loyalty and affection. It offered a version of shared intergenerational and inter-household responsibility writ small that contrasted with, but was also deeply responsive to, ideas about children's detrimental movements within the context of HIV.

RESIDENCE AND RELATIONSHIPS

77

In this chapter, I described a range of ways in which children and adults constructed ideals about good care. I demonstrated that the chronicity of poor health and poverty and the prominence of global health initiatives shaped ideals about and practices of good care, as well as the interdependence between adults and children. I view this chapter as a foundation for the remaining chapters, in which I will demonstrate, still further, how proximity and the nurturing of particular relationships in specific places—what I referred to in the introduction as “being closer”—served as a strategy that the children turned to when guardians became ill.